

1. Administrative & Study Identification

Full Study Title: A Study to Evaluate Nivolumab in Combination With Ipilimumab Versus Pemetrexed With Cisplatin or Carboplatin for Unresectable Pleural Mesothelioma in Chinese Participants
Short Title/Acronym: CheckMate 6DW (CA209-6DW) **Protocol Number:** CA209-6DW **NCT Number:** NCT05136677 **Posting ID:** 226792224995519156
Sponsor Name: Bristol Myers Squibb **Company:** Bristol Myers Squibb
Investigational Product: Nivolumab in combination with Ipilimumab
Phase of Development: Phase II (Phase ID: PHASE2) **Trial Status:** ACTIVE_NOT_RECRUITING (Active but not currently recruiting)
Medical Monitor: Bristol Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy and safety of the combination of nivolumab and ipilimumab versus standard-of-care chemotherapy (pemetrexed with cisplatin or carboplatin) as a first-line treatment in Chinese participants with unresectable malignant pleural mesothelioma (MPM). **Secondary Objectives:** To assess secondary efficacy endpoints including Progression-Free Survival (PFS), Objective Response Rate (ORR), Duration of Response (DOR), and to characterize the safety and tolerability profile of the combination immunotherapy in this specific demographic.

2.2 Background & Rationale To address the unmet medical need for effective, durable first-line therapies for Chinese patients with advanced, unresectable Malignant Pleural Mesothelioma (MPM). Global studies (e.g., CheckMate 743) have demonstrated the superiority of dual immune checkpoint inhibition (nivolumab + ipilimumab) over standard platinum-based chemotherapy. This study serves to bridge those clinical findings to the Chinese patient population, establishing clinical efficacy and safety locally.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Pemetrexed with Cisplatin or Carboplatin)
Blinding: Open-label **Randomization:** Randomized, Parallel Group
Phase Requirement: Trial must be in PHASE2 (Criteria Type: PHASE_REQUIREMENT)

3.2 Planned Enrollment & Duration Targeted Enrollment: 100 evaluable patients **Number of Sites:** Multiple sites across China

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histologically proven diagnosis of Malignant Pleural Mesothelioma (MPM) with determination of epithelioid vs non-epithelioid histology. 2. Must have advanced unresectable disease that is not amenable to therapy with curative intent (surgery with or without chemotherapy). 3. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 to 1. 4. Measurable disease. 5. Available tumor samples for centralized testing. *(Other protocol-defined inclusion criteria apply)*

4.2 Exclusion Criteria 1. Primitive peritoneal, pericardial, testis or tunica vaginalis mesothelioma. 2. Brain metastasis, except if surgically resected or treated with stereotaxic radiotherapy. 3. Prior therapy for MPM (including chemotherapy, radical pleuropneumectomy and non-palliative radiotherapy). *(Other protocol-defined exclusion criteria apply)*

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **Investigational Arm:** Nivolumab (IV) + Ipilimumab (IV) at specified dosing intervals (e.g., Nivolumab Q3W and Ipilimumab Q6W). * **Active Comparator Arm:** Pemetrexed (IV) in combination with either Cisplatin (Drug Preferred Name: cisplatin, Trade Name: Platinol, ATC Code: L01XA01, Source Level: 5) or Carboplatin (IV) administered every 3 weeks (Q3W) for up to 6 cycles. **Route of Administration:** Intravenous (IV) **Duration of Treatment:** Until disease progression, unacceptable toxicity, or maximum allowed treatment duration for immunotherapy.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care (anti-emetics, hydration for platinum doublet therapy), and palliative medications. **Prohibited:** Concurrent systemic anti-cancer therapies (chemotherapy, targeted therapy), systemic corticosteroids (>10 mg daily prednisone equivalent) unless used for treating immune-mediated adverse events, and live-attenuated vaccines.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Tumor Biopsy, Baseline Imaging (CT/MRI), Labs
Baseline	Day 0	Randomization, First Dose Administration, Safety Audit	Interim Every 3 to 6 Weeks Safety Labs,

Efficacy Assessment (Tumor Imaging Q6W), AE monitoring, PK sampling | | **End of Study** | Upon Progression | Final Vital Signs, Exit Interview, Treatment Discontinuation, Transition to Survival Follow-up |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Overall Survival (OS) – defined as the time from randomization to death from any cause.

7.2 Secondary & Exploratory Endpoints * Progression Free Survival (PFS) by modified RECIST (m-RECIST) by Investigator. * Objective Response Rate (ORR) by modified Response Evaluation Criteria in Solid Tumors (m-RECIST) by Investigator. * Incidence of Adverse Events (AEs).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring utilizing Common Terminology Criteria for Adverse Events (CTCAE). Specific tracking protocols are in place for immune-mediated AEs (e.g., pneumonitis, colitis, hepatitis). **Serious Adverse Events (SAEs):** Required to be reported to the sponsor's pharmacovigilance team within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** External, independent DMC tasked with periodic review of safety and unblinded efficacy data.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for all primary and secondary efficacy endpoints. Safety Set (all randomized patients receiving ≥ 1 dose of study drug) for safety analyses. **Sample Size Justification:** Appropriately powered to evaluate the consistency of the treatment effect (Hazard Ratio) in the Chinese subgroup compared to the global phase III pivotal trial. **Handling of Missing Data:** Survival data analyzed via Kaplan-Meier methodology with administrative censoring at the last known alive date.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Combination of routine onsite monitoring visits and centralized remote data monitoring throughout the trial's lifecycle. **Audit Trail:** Secure, 21 CFR Part 11-compliant Electronic Data Capture (eCRF) system featuring automated edit

checks, query generation (data clarification forms), and time-stamped audit trails.

11. Study Identifiers & Governance

Primary ID: CA209-6DW **Registry Number:** NCT05136677 **Posting ID:** 226792224995519156 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** CheckMate 6DW **Official Regulatory Title:** A Phase II, Randomized, Open-label Trial of Nivolumab in Combination With Ipilimumab Versus Pemetrexed With Cisplatin or Carboplatin as First-line Therapy for Unresectable Pleural Mesothelioma in Chinese Participants

12. Clinical Framework (Oncology/Mesothelioma Specific)

Disease Areas: Mesothelioma, Malignant **Name:** Mesothelioma **Preferred Name:** Mesothelioma **Preferred UMLS Name:** Malignant Mesothelioma **Semantic Type:** Neoplastic Process **Definition:** A tumor derived from mesothelial tissue (peritoneum, pleura, pericardium). It appears as broad sheets of cells, with some regions containing spindle-shaped, sarcoma-like cells and other regions showing adenomatous patterns. Pleural mesotheliomas have been linked to exposure to asbestos. (Dorland, 27th ed) **MeSH Code:** D008654 **SNOMED ID:** 62064005 **SNOMED Hierarchy:** * 1240414004|Malignant neoplasm|Malignant neoplasm (morphologic abnormality) * 115232000|Mesothelial neoplasm|Mesothelial neoplasm (morphologic abnormality) * 108369006|Neoplasm|Neoplasm (morphologic abnormality) **Medical Methodology:** Dual Immune Checkpoint Inhibition vs. Platinum-based doublet chemotherapy **Optimization Target:** Maximal tumor volume reduction and prolongation of Overall Survival

13. Study Procedures & Methodology

Management Pathway: Standard oncological pathway for advanced thoracic malignancies. **Education Protocol (HCP):** Mandated training on the identification, grading, and management of immune-related adverse events (irAEs). **Education Protocol (Patient):** Comprehensive onboarding regarding treatment scheduling, potential immunotherapy side effects, and strict AE reporting requirements. **Quality Audit Mechanism:** Centralized independent radiological review of scans and rigorous safety data reconciliation.

14. Observation & Follow-Up Schedule

Total Duration: Follow-up for survival (Up to 2-5 years post-randomization) **Onsite Visit Cadence:** Coordinated with dosing

cycles (Every 3 weeks for Nivolumab and Chemotherapy arms) **Remote Monitoring Frequency:** Survival follow-up phone calls every 12 weeks post-study drug discontinuation **Checklist Review Interval:** Prior to the initiation of every new treatment cycle

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]				Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]				